

Metropolitan General Hospital

PATIENT HEALTH RECORD & DISCHARGE SUMMARY

Patient Name: Arthur
Pendelton

MRN: #9842103

DOB: 05/14/1974

Sex: Male

Encounter Date:
09/09/2026

Attending Physician: Dr. Sarah Jenkins,
MD

Department: Internal Medicine & Cardiology

CHIEF COMPLAINT

Intermittent chest discomfort and shortness of breath upon exertion over the past 3 weeks.

HISTORY OF PRESENT ILLNESS (HPI)

The patient is a 52-year-old male who presents to the outpatient clinic reporting a 3-week history of mild, intermittent retrosternal chest tightness. The discomfort typically occurs during moderate physical exertion (e.g., climbing stairs) and resolves within 5 to 10 minutes with rest. He denies any associated diaphoresis, nausea, palpitations, or radiating pain to the left arm or jaw. Reports feeling fatigued lately but attributes it to a heavy work schedule. No prior cardiac history.

PAST MEDICAL HISTORY (PMH)

- **Essential Hypertension:** Diagnosed 2018, managed with Lisinopril.
- **Type 2 Diabetes Mellitus:** Diagnosed 2021, managed with Metformin.
- **Seasonal Allergic Rhinitis:** Controlled.

MEDICATIONS

- Lisinopril 10 mg PO daily
- Metformin 500 mg PO BID
- Cetirizine 10 mg PO PRN (allergies)

PHYSICAL EXAMINATION

- **General:** Alert, oriented x4, no acute distress.
- **Vitals:** BP 138/84 mmHg, HR 74 bpm, Temp 98.4°F (36.9°C), RR 16 breaths/min, SpO2 98% on room air.
- **Cardiovascular:** Regular rate and rhythm (RRR), normal S1 and S2, no murmurs, gallops, or rubs appreciated. Peripheral pulses intact (+2 bilaterally). No peripheral edema.
- **Respiratory:** Clear to auscultation bilaterally, no wheezes, rales, or rhonchi.
- **Abdomen:** Soft, non-tender, non-distended, bowel sounds normoactive.

DIAGNOSTIC RESULTS & LABORATORY FINDINGS (COLLECTED: 09/08/2026)

Test Panel	Parameter	Result	Reference Range / Status
Complete Blood Count (CBC)	WBC	6.8 x10 ³ /uL	4.5 - 11.0 (Normal)
	RBC	4.9 x10 ⁶ /uL	4.3 - 5.9 (Normal)
	Hemoglobin	14.2 g/dL	13.5 - 17.5 (Normal)
	Platelets	245 x10 ³ /uL	150 - 450 (Normal)
Comprehensive Metabolic Panel	Fasting Glucose	132 mg/dL	< 100 (Elevated)
	Creatinine	1.0 mg/dL	0.7 - 1.3 (Normal)
	eGFR	> 60 mL/min	> 60 (Normal)
Lipid Panel	Total Cholesterol	215 mg/dL	Borderline high
	Triglycerides	160 mg/dL	Borderline high
	LDL Cholesterol	135 mg/dL	Elevated
	HDL Cholesterol	48 mg/dL	Normal

Electrocardiogram (ECG): Normal sinus rhythm at 72 bpm. Normal axis. No acute ST-T wave abnormalities indicative of active ischemia.

ASSESSMENT / CLINICAL IMPRESSION

1. Atypical chest pain, low-to-intermediate pre-test probability of coronary artery disease (CAD).
2. Essential hypertension, currently suboptimally controlled.
3. Type 2 Diabetes Mellitus, mildly elevated fasting glucose.
4. Hyperlipidemia.

PLAN & RECOMMENDATIONS

- **Cardiology Referral:** Scheduled for an outpatient stress echocardiography within 2 weeks.
- **Medication Adjustment:** Increase Lisinopril to 20 mg PO daily for better blood pressure control. Continue Metformin 500 mg BID. Initiate Atorvastatin 20 mg PO daily for hyperlipidemia management.
- **Lifestyle Modifications:** Heart-healthy diet (low sodium, low saturated fats), moderate aerobic exercise (30 mins/day, 5 days/week) pending stress test results.
- **Patient Education:** Warned about red flag symptoms (severe chest pain, sudden shortness of breath, dizziness) requiring immediate evaluation at the Emergency Department.
- **Follow-up:** Return to clinic in 4 weeks or sooner if symptoms worsen.